

1. Administrative & Study Identification

Full Study Title: An Open-label, Single-arm, Multicenter, Phase 1 Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Preliminary Efficacy of BMS-986489 (BMS-986012 + Nivolumab Fixed Dose Combination) in Chinese Participants With Relapsed/Refractory Small Cell Lung Cancer **Short Title/Acronym:** Phase 1 Study of BMS-986489 in Relapsed/Refractory SCLC **Protocol Number:** CA245-0002 **NCT Number:** NCT06799286 **Sponsor Name:** Bristol-Myers Squibb **Investigational Product:** BMS-986489 (Atigotatug + Nivolumab Fixed-dose Combination) **Phase of Development:** Phase 1 **Medical Monitor:** Bristol-Myers Squibb Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, pharmacokinetics, and preliminary efficacy of BMS-986489 in Chinese participants with relapsed/refractory small cell lung cancer (SCLC). **Secondary Objectives:** To assess Objective Response Rate (ORR), Duration of Response (DOR) using RECIST v1.1, Progression-Free Survival (PFS), and Overall Survival (OS).

2.2 Background & Rationale To address the unmet medical need and lack of effective treatments in patients with relapsed or refractory SCLC. This study aims to generate safety, pharmacokinetic, and preliminary efficacy data for BMS-986489—a novel fixed-dose combination of Atigotatug (anti-fucosyl-GM1) and Nivolumab (anti-PD-1)—specifically within the Chinese patient population, offering a potential new immunotherapy approach for recurrent disease.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 10 **Number of Sites:** Multicenter (China) **Estimated Study Start:** 07-May-2025 **Estimated Primary Completion:** 26-May-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants must have histologically or cytologically documented SCLC with either limited or extensive

disease stage at the initial diagnosis. 2. Must have received at least one prior line of systemic therapy (e.g., for extensive stage, at least one line of platinum-based systemic therapy with/without immunotherapy). 3. Progressed or recurred within 6 months after treatment of curative surgical resection, systemic therapy, or radiotherapy, or been intolerant to the prior systemic therapy.

4.2 Exclusion Criteria 1. Untreated symptomatic CNS metastases or leptomeningeal disease. 2. Pleural effusion which cannot be controlled with appropriate interventions or malignancy-related superior vena cava syndrome. 3. Active, known or suspected, autoimmune disease; concurrent malignancy requiring treatment; or prior treatment with an anti-fuc-GM1 therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986489 (Atigotatug + Nivolumab Fixed-Dose Combination) per protocol-specified dose escalation/expansion rules. **Route of Administration:** Intravenous (IV) parenteral. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Medications allowed for management of co-morbidities and standard supportive care. **Prohibited:** Prior treatment with an anti-fuc-GM1 therapy or any other drug specifically targeting fuc-GM1; concurrent anti-tumor therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
:---	:---	:---
:---	:---	Screening Day -14 to 0 Informed Consent, Medical History, Tumor Imaging, Safety Labs
		Baseline Day 0 Enrollment, First Dose of BMS-986489, PK Sampling
Week [X]	\$\pm\$ [Y]	Interim Safety Labs, PK/PD Sampling, Tumor Assessment (RECIST v1.1), AE Monitoring
		End of Study Week [X] Final Vital Signs, Exit Interview, Follow-up for Survival and Progression

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of Adverse Events (AEs), Serious Adverse Events (SAEs), and Dose-Limiting Toxicities (DLTs) to establish safety and tolerability of BMS-986489. **Measurement Method:** Clinical assessments, physical examinations, and Central/Local Laboratory Analysis.

7.2 Secondary & Exploratory Endpoints Objective Response Rate (ORR): Achievement of best response of complete response (CR) or partial response (PR) using Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 criteria, assessed by the investigator. **Duration of Response (DOR):** Time from first response (CR or PR) to first documented disease progression by investigator or death, whichever occurs first.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of adverse events and immune-related AEs (irAEs) via physical exams and safety labs. **Serious Adverse Events (SAEs):** Required reporting within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal and/or external safety review committee to evaluate dose-escalation decisions and overall safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for efficacy; Safety Set for all participants receiving at least one dose of study medication. **Sample Size Justification:** Sample size will be driven by the requirements of Phase 1 dose-escalation guidelines to adequately evaluate DLTs and confirm the recommended Phase 2 dose (RP2D). **Handling of Missing Data:** For Time-to-Event variables (e.g., DOR), subjects without an event will be censored on the date of their last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to ensure data integrity and protocol compliance. **Audit Trail:** Standard tracking of eCRF entries, data clarification forms, and automated audit trails in the clinical database.

11. Study Identifiers & Governance

Primary ID: NCT06799286 **Registry Number:** NCT06799286 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** An Open-label, Single-arm, Multicenter, Phase 1 Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Preliminary Efficacy of BMS-986489 (BMS-986012 + Nivolumab Fixed Dose Combination) in Chinese Participants With Relapsed/Refractory Small Cell Lung Cancer **Official Regulatory Title:** An Open-label, Single-arm, Multicenter, Phase 1 Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Preliminary Efficacy of BMS-986489

12. Clinical Framework (Oncology/SCLC Specific)

Primary Condition: Relapsed/Refractory Small Cell Lung Cancer (SCLC) **Diagnosis Threshold:** Histologically or cytologically documented SCLC progressed after ≥ 1 prior line of systemic therapy **Medical Methodology:** Investigational Immunotherapy (Anti-fuc-GM1 + Anti-PD-1) **Optimization Target:** Objective Tumor Response (CR/PR)

13. Study Procedures & Methodology

Management Pathway: Standard clinical oncology pathway for Relapsed/Refractory SCLC **Education Protocol (HCP):** Protocol training, dose modification guidelines, and immune-related AE (irAE) management **Education Protocol (Patient):** Informed consent, onsite treatment education, and self-reporting of immune-mediated symptoms **Quality Audit Mechanism:** Independent clinical monitoring, RECIST v1.1 centralized/local review verification

14. Observation & Follow-Up Schedule

Total Duration: Continued until disease progression or unacceptable toxicity **Onsite Visit Cadence:** Aligned with infusion schedule (e.g., every 2 to 3 weeks) **Remote Monitoring Frequency:** Continuous safety and survival follow-up **Checklist Review Interval:** Regular tumor assessments per RECIST v1.1 (e.g., every 6 to 8 weeks)

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				